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Head Start Program Self-Assessment Checklist

Annual self-assessment checklist for Head Start programs.
Covers ERSEA, health services, family engagement, education,
and fiscal compliance areas.

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- ☐ Confirm attendance dates match room records.
- ☐ Mark ratio notes that support licensing review.
- ☐ Attach subsidy claim evidence before filing.
- ☐ Keep guardian and pickup updates with the child record.

How to Use This Checklist

Assign each section to the relevant program manager or coordinator to gather documentation before the assessment meeting. Hold the full self-assessment with stakeholders present to review findings together. Record both strengths and areas for improvement — an honest assessment with documented weaknesses is better than a superficial one. All findings and the resulting Quality Improvement Plan must be submitted to the governing body and policy council.

Section 1: ERSEA — Eligibility, Recruitment, Selection, Enrollment, and Attendance

Eligibility Documentation ★

- ☐ Income eligibility documentation is on file for all currently enrolled children
- ☐ Documentation type matches the income verification method used (pay stubs, tax return, benefit letter, homeless documentation, etc.)
- ☐ Over-income children enrolled under a waiver (up to 35% of enrollment): waiver documentation is current and justified
- ☐ Foster children and children who are homeless or in the child welfare system are documented according to their categorical eligibility status
- ☐ Eligibility is re-determined at least annually — all re-determination records are current
- ☐ Eligibility files are organized and retrievable within a reasonable timeframe for federal review

Recruitment

- ☐ A written recruitment plan is in place for the current program year
- ☐ Recruitment efforts are documented (outreach activities, materials distributed, community partnerships engaged)
- ☐ Recruitment reaches the hardest-to-serve families in your service area (document your strategies for reaching these families)
- ☐ Waiting list is maintained and managed according to your selection criteria
- ☐ Community need data supports the enrollment population being served

Selection

- ☐ Selection criteria are documented and consistently applied
- ☐ Priority enrollment categories are followed: children experiencing homelessness, children in foster care, children from families below the poverty line
- ☐ Selection decisions are documented with reason for selection (priority category applied)
- ☐ No child was selected outside priority criteria while higher-priority eligible children remained on the waiting list without documented reason

Enrollment and Attendance ★

- ☐ Actual enrollment meets or exceeds funded enrollment level for the current reporting period
 - ☐ Enrollment forms are complete and signed for all children
 - ☐ Daily attendance is recorded for each enrolled child at each service session
 - ☐ Attendance tracking system can produce a record of attendance for any child for any date range within 24 hours
 - ☐ Chronic absence patterns (missing more than 20% of scheduled days) are identified and family services staff are engaged
 - ☐ Children who have missed [X] consecutive days have been contacted by family services staff; contact attempts are documented
 - ☐ Enrollment data matches the count used in ERSEA eligibility files
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Section 2: Education and Child Development

Individualized Learning

- ☐ Individual lesson plans or learning goals are developed for each child based on assessment data
- ☐ Ongoing child assessment data (Teaching Strategies GOLD, DRDP, or program-selected tool) is collected at required intervals
- ☐ Assessment data is used to inform classroom practice — evidence of this is visible in lesson plans and teacher documentation
- ☐ For dual language learners: language support strategies are individualized based on the child's home language and English proficiency
- ☐ Transitions to kindergarten are planned individually for each child aging out of the program

Developmental Screening ★

- ☐ Developmental screening (ASQ, PEDS, or other state-approved tool) is completed within 45 days of enrollment for all newly enrolled children
- ☐ Results are documented in each child's file
- ☐ Families received and signed documentation of screening results
- ☐ Children identified with potential developmental concerns were referred for further evaluation
- ☐ Referrals were followed up — documentation of follow-up is on file

Children with Disabilities

- ☐ Children with disabilities make up at least 10% of total enrollment (this is a federal requirement)
- ☐ All children with an active IFSP or IEP have a copy of the current plan on file
- ☐ Program is participating in IFSP/IEP team meetings and service coordination as required
- ☐ Disability services coordinator (or equivalent role) is identified and current in the role
- ☐ Reasonable accommodations and services outlined in the IFSP/IEP are being implemented

Classroom Quality

- ☐ Structured CLASS observation or other program quality assessment tool has been conducted in each classroom within the past program year
 - ☐ Results are reviewed with teaching staff and inform professional development planning
 - ☐ Teacher-child interactions reflect responsive and intentional teaching practices
 - ☐ Learning environments are organized, safe, and support exploration across all domains (cognitive, social-emotional, language, physical)
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Section 3: Health Services

Health Records ★

- ☐ A complete health history form is on file for every enrolled child
- ☐ Physical examination documentation is on file (completed within 90 days before or 30 days after enrollment, per Head Start requirements — confirm current standard)
- ☐ Dental exam documentation is on file for all children who are age-eligible (typically 3 years and older)
- ☐ Immunization records are current and on file per the state immunization schedule
- ☐ Children who are not up to date on immunizations have a documented exception (medical, religious/philosophical where permitted) or an active catch-up schedule in place

Health Screenings ★

- ☐ Vision screening completed within 45 days of enrollment (or referral documented with results pending)
- ☐ Hearing screening completed within 45 days of enrollment (or referral documented with results pending)
- ☐ Lead screening — documentation that blood lead level has been checked per state guidance is on file or referral made
- ☐ BMI/height and weight measurements recorded at required intervals
- ☐ Children identified with health concerns received follow-up referrals; follow-up is documented

Oral Health

- ☐ Dental exam completed for all age-eligible children
- ☐ Children with identified dental treatment needs: follow-up with dental care documented
- ☐ Oral health education provided to families (document when and how)
- ☐ Toothbrushing supplies available and toothbrushing conducted in classrooms (for full-day programs)

Mental Health

- ☐ Mental health consultant is under contract or available to the program
- ☐ Mental health consultations documented for children referred
- ☐ Staff have received training on social-emotional development and trauma-informed practices within the past program year
- ☐ Expulsion and suspension policy is consistent with Head Start requirements (chronic absence should be addressed before formal action; expulsion should be rare and documented)

Nutrition

- ☐ Meals served meet CACFP meal pattern requirements for the age groups served
 - ☐ Special dietary needs are documented and accommodated with written plans
 - ☐ Nutrition education provided to families (document activities)
 - ☐ CACFP claim records are complete and current
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Section 4: Family Engagement

Family Partnership Agreements ★

- ☐ A Family Partnership Agreement (or family goal-setting document) is developed with every family within 90 days of enrollment
- ☐ Family Partnership Agreements are updated at least annually
- ☐ Goals in the agreement are specific, family-identified, and meaningful — not generic forms completed without family input
- ☐ Progress toward goals is documented in subsequent contact notes

Home Visits (if applicable to program option)

- ☐ Required home visits are completed for all families per the Head Start Performance Standards for your service option
- ☐ Home visit dates, duration, and topics are documented in family records
- ☐ Home visits are not being scheduled and then completed on paper — verify actual completion against documentation

Parent Committee and Policy Council

- ☐ Parent committee (at the site level) has met the required number of times this program year
- ☐ Meeting dates, attendance, and minutes are documented and on file
- ☐ Policy Council has met the required number of times this program year
- ☐ Policy Council meeting minutes are documented, signed, and retained
- ☐ Policy Council members have been properly elected — election documentation is on file
- ☐ Policy Council has been provided with all required documents for review (budget, financial reports, self-assessment, Quality Improvement Plan)
- ☐ Policy Council has had meaningful opportunity to approve or disapprove items requiring their approval

Parent Activities and Volunteer Engagement

- ☐ Parent education and engagement activities have been offered this program year (document dates, topics, attendance)
 - ☐ Volunteer opportunities are available to families and are documented
 - ☐ Family transitions into employment, education, and training are tracked where families have consented to share this information
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Section 5: Governance

Governing Body

- ☐ Governing body (board of directors or equivalent) has met at least quarterly
- ☐ Meeting minutes are complete, signed, and retained
- ☐ Governing body has received and reviewed all required reports (financial, enrollment, self-assessment findings, compliance data)
- ☐ Governing body composition meets requirements (parent representation, community members, etc.)
- ☐ Governing body has approved required items including budget, audit, and program changes

Policy Council

- ☐ Policy Council has approved the program's budget
- ☐ Policy Council has reviewed the annual self-assessment findings
- ☐ Policy Council has approved the Quality Improvement Plan
- ☐ Policy Council has had the opportunity to participate in hiring the program director (or higher-level staff, per your program's procedures)

Organizational Documentation

- ☐ Current organizational chart is on file and reflects actual staffing
 - ☐ Position descriptions are current for all funded positions
 - ☐ Delegate agency agreements (if applicable) are current and signed
 - ☐ Memoranda of understanding with community partners are current and on file
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Section 6: Fiscal Management

Budget and Financial Oversight

- ☐ Federal grant budget has been approved by the governing body and policy council for the current budget period
- ☐ Program is operating within the approved budget — no unapproved significant budget modifications
- ☐ Non-federal match requirement is being met; documentation of in-kind contributions is on file
- ☐ Annual audit has been completed (if applicable) and any audit findings have an active corrective action plan

ERSEA and Payment Records Alignment ★

- ☐ Eligibility documentation on file matches the children for whom the program is receiving federal funding
 - ☐ Enrollment counts reported in PIR and federal data systems match actual enrollment records
 - ☐ Any changes to enrollment counts reported correctly and in a timely manner
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Annual Self-Assessment Process Steps

Follow this sequence each year to ensure the self-assessment meets federal requirements.

Step 1: Assemble the Self-Assessment Team

The team should include:

- ☐ Program director
- ☐ Education coordinator
- ☐ Family services coordinator
- ☐ Health coordinator
- ☐ Fiscal manager
- ☐ At least two current or recent parent representatives
- ☐ Policy Council representative
- ☐ Governing body representative

Identify team members and confirm participation at least 8 weeks before the assessment.

Step 2: Collect Data (6-8 weeks before assessment meeting)

Assign each coordinator to pull documentation for their service area using the checklists above. Data to collect includes:

- ☐ Enrollment and attendance records
- ☐ Health screening completion rates
- ☐ Developmental screening rates and follow-up rates
- ☐ Family Partnership Agreement completion rates
- ☐ Parent committee and Policy Council meeting records
- ☐ Training and professional development records for all staff
- ☐ Classroom observation data
- ☐ Prior year Quality Improvement Plan — status of each improvement item

Step 3: Conduct the Assessment Meeting

- ☐ All required stakeholders present or represented
- ☐ Each service area presented with strengths identified and documented
- ☐ Each service area reviewed for gaps and areas for improvement; gaps documented honestly
- ☐ Root causes discussed for any area that did not meet the standard
- ☐ Preliminary improvement priorities identified

Step 4: Document Findings

- ☐ Written findings document completed summarizing results of each section
- ☐ Strengths and areas for improvement recorded for each service area
- ☐ Document is signed or acknowledged by the team lead and program director

Step 5: Develop the Quality Improvement Plan

The QIP must be specific, not general. For each identified area for improvement:

- ☐ Specific goal stated (what will be different)
- ☐ Strategy or action steps described
- ☐ Person responsible identified by name and role
- ☐ Timeline established with intermediate checkpoints
- ☐ How progress will be measured defined

Step 6: Submit to Governing Body and Policy Council

- ☐ Self-assessment findings submitted to governing body for review at their next scheduled meeting
- ☐ Self-assessment findings submitted to Policy Council for review at their next scheduled meeting
- ☐ Quality Improvement Plan approved by governing body
- ☐ Quality Improvement Plan approved by Policy Council
- ☐ Approval documented in meeting minutes

Step 7: Implement and Monitor

- ☐ QIP is distributed to all responsible staff
- ☐ Progress is reviewed at least quarterly against the QIP
- ☐ Governing body and Policy Council receive quarterly progress updates
- ☐ Prior year QIP status is the first section reviewed at next year's self-assessment

